

DRAFT QUALITY ASSURANCE SURVEILLANCE PLAN (QASP)**Department of Homeland Security (DHS)****Office of Health Security (OHS)****Medical Services Strategic Sourcing Vehicle (SSV) for Persons in Custody (PIC) and Related Services****1. INTRODUCTION**

This Quality Assurance Surveillance Plan (QASP) is founded on the principle that the awarded medical service provider, and not the Government, is responsible for the day-to-day delivery and operation of those in-scope medical services at designated performance locations which includes, but is not limited to, DHS detention centers, ports of entry, mobile units, emergency shelters, and other performance locations. The medical service provider is also responsible for all management and quality control actions pursuant to the requirements outlined in the resulting work statement.

The role of the Government in quality assurance is to ensure satisfactory performance is achieved and maintained. The medical service provider shall develop a comprehensive program of inspections and monitoring actions and document its approach in a Quality Control Plan (QCP). The medical service provider's QCP, upon approval by the Government, will be made a part of the resultant contract award.

This QASP is designed to provide an effective surveillance method to monitor the medical service provider's performance relative to the requirements listed in the solicitation Statement of Objectives (SOO) and resulting work statement. The QASP illustrates the systematic method the Government (or its designated representative) will use to evaluate the services the medical service provider is required to furnish.

This QASP is based on the premise that the Government will validate the medical service provider's compliance with DHS-mandated quality standards to ensure the successful delivery of in-scope medical services at DHS detention centers, ports of entry, mobile units, emergency shelters, and other performance locations. Performance standards encompass all facets of medical service delivery, including safety, health, legal rights, performance location, and records management, etc. Effective management by the medical service provider, along with the implementation of an approved QCP, will ensure that performance is within acceptable quality levels.

2. DEFINITIONS

Acceptable Quality Level: The minimum level of quality that will be accepted by the Government to meet the performance standard.

Contract Task Order Manager/Monitor (CTOM): A regional and/or sector-level government appointed COR-certified individual in the field where medical unit operations exists, who is

appointed by the CO to work with the COR Headquarters and CO in the day-to-day technical surveillance monitoring, execution, and administration of the contract. The CTOM coordinate with designated Field Office Leadership (OFLs) and On-Duty Supervisors (ODS'), where these roles are not empowered to make any contractual commitments or to authorize any changes on the Government's behalf. Any changes that the contractor deems may affect contract price, terms, or conditions shall be referred to the CO for action, as supported and facilitated by the COR/CTOM according to the limitations of the appointment letters.

Contracting Officer: A person duly appointed with the authority to enter into contracts and make related determination and findings on behalf of the Government. The contracting officer (CO) has a written warrant, which sets forth limitations of authority. The CO assures the contractor receives impartial, fair and equitable treatment under the contract. The CO is ultimately responsible for the final determination of the adequacy of the contractor's performance.

Contracting Officer's Representative (COR): The COR interacts with the service provider to inspect and accept services/work performed in accordance with the technical standards prescribed in the agreement. The CO issues a written memorandum that appoints the COR. Other individuals may be designated to assist in the inspection and quality assurance surveillance activities. The COR is not empowered to make any contractual commitments or to authorize any changes on the Government's behalf. Any changes that the contractor deems may affect contract price, terms, or conditions shall be referred to the CO for action.

Deduction: Funds may be deducted from a monthly invoice for a failure to perform required services, or if the same deficiency continues to occur. The medical service provider will be notified immediately if such a situation arises. The CO in consultation with the COR and general counsel will determine the amount of the deduction. Amounts deducted are not recoverable.

Functional Area: A logical grouping of performance standards.

Government Technical Monitor: Government Technical Monitor (GTM): The Contracting Officer may appoint Government Technical Monitors (GTM) to assist the COR in monitoring a contractor's performance. A GTM must be a government employee. Responsibilities and duties are outlined at the component level.

Licensed Independent Provider (LIP): a Healthcare professional who is permitted by law to provide patient care services without direction or supervision, within the scope of their license and consistent with the clinical privileges granted.

Measures: The method for evaluating compliance with the standards.

Operational Field Liaison (OFL) and On-Duty Supervisors (ODS): A regional-level government appointed individual who is designated by the DHS authorized official in charge of the designated work area (e.g. Patrol Agent in Charge, Field Office Director, etc.), to work with the COR/CTOM to assist in any related field inspections, audits, and coordination at the sites, and for any day-to-day operations from which the contract supports. The OFL may be supported by one or more designated ODS to provide coverage for each work shift.

Performance Requirements Summary (PRS): The PRS communicates what the Government intends to qualitatively inspect. The PRS identifies performance standards grouped into functional areas, and quality levels essential for successful performance of each requirement. The PRS is used by the Government when conducting quality assurance surveillance to guide through the inspection and review processes. The PRS is based on laws, rules, regulations, and nationally recognized standards of care to include, but not limited to:

- a. The Immigration and Nationality Act (INA) as amended, 8 U.S.C. 1101 et seq.
- b. Homeland Security Act of 2002 (HSA), as amended
- c. Title 8 Code of Federal Regulations (CFR), as amended
- d. Title 6, United States Code (U.S.C.) § 350, "Work Force Health and Medical Support", as amended
- d. Occupational Safety and Health Administration (OSHA), as applicable
- e. Prison Rape Elimination Act (PREA)
- f. State Welfare Codes, as applicable
- g. State Licensing Boards, as applicable to all labor categories where DHS operates
- h. State Departments of Health or other governing entities as applicable
- i. National Commission on Correctional Health Care Standards (NCCHC), where applicable
- j. American Correctional Association (ACA), where applicable
- k. ICE Performance Based National Detention Standards (PBNDS) and associated revisions, as established for each performance location
- l. ICE Family Residential Standards (ICE FRS) - Residential and Family Residential Facilities, where applicable
- m. IHSC Medical By-Laws
- n. CBP and ICE Joint Intake Center (JIC) Reporting Guidance
- p. CBP National Standards on Transport, Escort, Detention, and Search (TEDS)
- q. DHS and DHS Component Codes of Conduct, Policy, Directives, Issuances, Guides, and Memoranda relating to care-in-custody standards
- r. National medical professional society and specialty organization standards

The Government reserves the right to adjust this list at any time based on changes in mission, health care needs, legislation, or other circumstances that dictate changes in practice. The contractor will be in compliance with the above referenced documents for the duration of the awarded contract.

Performance Standards: The performance standards are established within this QASP. Further, additional performance standards may be established at the task order level based on task order level requirements.

Program Manager: Responsible for overseeing and directing work, ensuring compliance with federal regulations, effective delivery of requirements and deliverables, identifying risks, and managing resources. Monitors funding and expenditures and adherence to cost, schedule, and performance baselines. Advise COR on approval of invoices.

Withholding: Amount of monthly invoice payment withheld pending correction of a deficiency. See below PRS for information on the percentages of an invoice amount that may be withheld

for each functional area. Funds withheld from payment are recoverable (See Sections 7 and 8) if the COR and CO confirm resolution or correction and should be included in the next month's invoice.

3. QUALITY CONTROL PLAN

The medical service provider shall develop, implement, and maintain a QCP that illustrates the methods it will use to review its performance to ensure it conforms to the performance requirements (see below for a summary list of performance requirements). Such reviews shall be performed by the medical service provider to validate its operations and assures the Government that the services meet the performance standards.

The medical service provider's QCP shall include monitoring methods that ensure and demonstrate its compliance with the performance standards. This includes inspection methods and schedules that are consistent with the regular reviews conducted by the Government. The medical service provider will execute and document all QCP activities, compliance status, and performance outcomes. The reports and other results generated by the medical service provider's QCP activities will be provided to the COR as requested.

The medical service provider will define and implement monthly quality assurance reviews and internal audit procedures in a QCP and review the QASP metrics/performance. The medical service provider will execute and document the results of such reviews and audits, implement a corrective action plan on areas found deficient in performance, and ensure that all documentation (e.g., internal audit reports) related to them are available to the COR and the CO at any time during the awarded contract performance.

The medical service provider is encouraged not to limit its inspection to only the processes outlined in the applicable standards represented above within the PRS definition; however, certain key documents shall be produced by the medical service provider to ensure that the services meet the performance standards. The medical service provider shall develop and implement a program that addresses the specific requirement of each standard and the means it will use to document compliance. Some of the documentation that shall be generated and made available to the COR for inspection are listed below. The list is intended as illustrative and is not all-inclusive.

- Written policies and procedures to implement and assess operational requirements of the standard
- Documentation and record keeping to ensure ongoing operational compliance with the standards (e.g., inventories, logbooks, register of receipts, reports, etc.)
- Staff training, license, and certification records to ensure ongoing medical quality compliance with the standards and according to agency/component data/deliverable requirements
- Contract discrepancy reports (CDRs)
- Investigative reports
- Electronic medical records (EMR)
- Records of investigative actions taken

- Equipment inspections
- System tests and evaluation

4. METHODS OF SURVEILLANCE

The Government will monitor the medical service provider's compliance with the performance standards using a variety of methods. In addition, the Government may conduct additional routine, follow-up, or unscheduled ad hoc inspections as necessary (for instance, as a result of unusual incidents or data reflected in routine monitoring). The Government may also maintain an on-site presence in some performance locations to conduct more regular or frequent monitoring. Inspections and monitoring may involve direct observation of medical service conditions and operations, review of documentation (including QCP reports), and/or interviews of awarded contractor personnel and persons in custody.

4.1 Documentation Requirements: The medical service provider shall develop and maintain all documentation as prescribed in the requisite standards (e.g., post logs, policies, and records of corrective actions). In addition to the documentation prescribed by the standards, the medical service provider shall also develop and maintain documentation that demonstrates the results of its own inspections as prescribed in its QCP. The Government may review 100% of the documents, or a representative sample, at any point during the period of performance.

5. FUNCTIONAL PERFORMANCE AREAS AND STANDARDS

To facilitate the performance review process, the required performance standards are organized into functional areas. Each functional area represents a proportionate share (i.e., weight) of the monthly invoice amount payable to the service provider based on meeting the performance standards. Payment withholdings and deductions will be based on these percentages and weights applied to the overall monthly invoice.

The Government may, consistent with the scope of the agreement, unilaterally change the functional areas and associated standards affiliated with a specific functional area. The Government reserves the right to develop and implement new and/or tailor inspection techniques and instructions at any time during performance without notice to the medical service provide so long as the standards are not more stringent than those being replaced. Where the standards are more stringent than those being replaced, the Government will engage the vendor.

6. FAILURE TO MEET PERFORMANCE STANDARDS

Performance of medical services in conformance with the PRS standards is essential for the medical service provider to receive full reimbursement for in-scope services provided. The CO may take withholdings or deductions against the monthly invoices for unsatisfactory performance documented through surveillance of the medical service provider's activities gained through site inspections, reviews of documentation (including monthly QCP reports), interviews and other

feedback. As a result of its surveillance, the medical service provider will be assigned the following rating relative to each performance standard:

Rating	Description
Acceptable	Based on the measures, the performance standard is demonstrated.
At-Risk	Based on the measures, compliance with most of the attributes of the performance standard is demonstrated or observed with some area(s) needing improvement. There are no critical areas of unacceptable performance.
Deficient	Based on the performance measures, the majority of a performance standard's attributes are not met.

Using the above standards as a guide, the CO, based on a documented assessment prepared and provided for by the COR, will implement adjustments to the medical service provider's monthly invoice as prescribed below.

Rather than withholding funds until a deficiency is corrected, there may be times when an event or a deficiency is so egregious that the Government **deducts** (vs. "withholds") amounts from the medical service provider's monthly invoice. This may happen when a significant event occurs, when a particular deficiency is noted multiple times without correction, or when the medical service provider has failed to take timely action on a deficiency about which he was properly and timely notified.

The amount deducted will be consistent with the relative weight of the functional performance area where the deficiency was noted. The deduction may be a one-time event or may continue until the medical service provider has either corrected the deficiency or made substantial progress in the correction. Further, a deficiency found in one functional area may tie into another.

Identified deficiencies and application of resulting deductions do not impact in any way the Government's ability to take further action consistent with the nature of the deficiency. This may include for example, issuance of a cure notice and/or potential award termination.

7. NOTIFICATIONS

- (a) Based on inspection of the medical service provider's performance, the COR will document instances of deficient or at-risk performance (e.g., non-compliance with the standard) using the CDR. To the extent practicable, issues should be resolved informally, with the COR and medical service provider working together. When documentation of an issue or deficiency is required, the procedures set forth in this section will be followed. The COR will maintain a record of all open and resolved CDRs.
- (b) When a CDR is required to document performance issues, it will be submitted to the medical service provider with a date when a response is due. Upon receipt of a CDR, the medical service provider shall immediately assess the situation and either correct the deficiency as quickly as possible or prepare a corrective action plan. In either event, the service provider

shall return the CDR with the action planned or taken noted. After the COR reviews the medical service provider's response to the CDR including its planned remedy or corrective action taken, the COR will either accept the plan or correction or reject the correction or plan for revision and provide an explanation. This process should take no more than one week. The CDR shall not be used as a substitute for quality control by the medical service provider. Based on the nature of the identified deficiency, a more expedited process timeline may be established.

Note: in an emergency situation, a CDR may not be issued until an investigation has been completed.

- (c) If the COR concludes that the deficient or at-risk performance warrants a withholding or deduction, the COR will include the CDR in its monthly report, with a copy to the CO. The CDR will be accompanied by the COR's investigation report and written recommendation for any withholding or deduction. The CO will consider the COR's recommendation and forward the CDR along with any relevant supporting information to the medical service provider to confirm or further discuss the prospective cure, including the Government's proposed course of action. As described in section 7 above, portions of the monthly invoice amount may be withheld until such time as the corrective action is completed, *or* a deduction may be taken.
- (d) Following receipt of the medical service provider's notification that the correction has been made, a reinspection may take place. Based upon the findings, a recommendation will be made by the COR that the CO continue to withhold a proportionate share of the payment until the correction is made or accept the correction as final and release the full amount withheld for that issue. If a deduction was made for the infraction, the funds are not recoverable.
- (e) If funds have been withheld and the award, or awarded task order is terminated, those funds will not be released. The medical service provider may only receive withheld payments upon successful correction of an instance of non-compliance. Further, the service provider is not relieved of full performance of the required services hereunder; termination may occur, upon adequate notice from the Government based upon any one instance, or failure to remedy deficient performance, even if a deduction was previously taken for any inadequate performance.

8. CONTINUITY OF OPERATIONS PLAN (COOP)

Prepare and submit a Continuity of Operations Plan (COOP) to the Government. The COOP shall be due sixty (60) calendar days after the date of task order award and will be updated on an annual basis. The COOP shall document contractor plans and procedures to maintain support of the contract during an emergency at both the site specific and program level. The COOP, at a minimum, shall include the following:

- (a) A description of the contractor's emergency management procedures and policies;
- (b) A description of how the contractor will account for their employees during an emergency;
- (c) How the contractor will communicate with the Government during emergencies; and
- (d) A list of primary and alternate contractor points of contact, each with primary and alternate a) Telephone numbers, b) E-mail addresses.

Individual site COOPs shall be activated immediately after determining that an emergency has occurred and shall be operational within 24 hours of activation or as directed by the Government. The plan shall be sustainable until the emergency is resolved, and normal conditions are restored, or the contract is terminated, whichever comes first. In case of a life-threatening emergency, the COR shall immediately contact the contractor Project Manager to ascertain the status of any contractor personnel who were in operational spaces affected by the emergency. When any disruption of normal, daily operations occurs, the contractor Project Manager and the COR shall promptly open an effective means of communication and verify:

- (a) Key points of contact (Government and contractor);
- (b) Temporary performance locations (alternate office spaces, telework, virtual offices, etc.);
- (c) Means of communication available under the circumstances (e.g., email, telephone, courier, etc.);
- (d) Essential contractor work products expected to be continued, by priority; and
- (e) Methodology to document hours worked by contract staff.

The Government and contractor Project Manager shall make use of the resources and tools available to continue contracted functions to the maximum extent possible under emergency circumstances. Contractors shall obtain approval from the CO prior to incurring costs over and above those allowed for under the terms of this contract. Regardless of contract type, and performance location, contractors performing work in support of authorized tasks within the scope of their contract shall charge those hours accurately in accordance with the terms of this contract.

9. TRANSITION-IN AND -OUT PLAN

- (a) The medical service provider is responsible for the transition of all activities identified in this document and resulting work statement. The transition-in phase shall identify those actions, plans, procedures, and timelines necessary to ensure a smooth transition from contract start date to full operational status by the contractor. The contractor's transition-in shall be accomplished as expeditiously as possible, in accordance with the terms set forth in the task orders. The transition-in process shall not adversely impact the work being done by the outgoing contractor, and it shall be conducted in a manner consistent with safe operation requirements. All activities must be completed during transition periods.
- (b) The transition-in plan shall be approved by the COR and describe the contractor's process for transitioning from the incumbent with no disruption in operational services.
- (c) The transition-out phase shall identify those actions, plans, procedures, and timelines necessary to ensure a smooth transition to a new contractor from start date to full operational status by the new contractor. If the contractor is not awarded a follow-on contract, or if their performance is deemed unsatisfactory and a new vendor is awarded the contract, the

contractor shall be responsible for the transition-out of all technical activities identified in this document and resulting work statement during the final, awarded period of performance.

- (d) The incumbent contractor agrees to cooperate with the incoming contractor to minimize the disruption of services during the change-over of operations. The incumbent contractor shall continue to perform contract requirements during the period between the initial decision date and completion of the phase-out period. The incumbent contractor shall fully support the transition of all requirements to any successor to ensure no disruption in operational services.
- (e) The incumbent contractor, the incoming contractor, and the CORs shall plan contract change-over procedures within ten (10) calendar days of a follow-on contract award, to consider, at a minimum, the following: the time to effect, the tasks to be performed, and the information and or data to be exchanged.
- (f) In the event the contractor, the incoming contractor, and the COR cannot agree on change-over procedures, all parties shall comply with the terms and procedures as established by the CO.
- (g) The transitioning out (incumbent) contractor shall submit the Transition-out Plan 15 days following official notification of a transition out period. The transition-out plan shall be approved by the COR. The contractor shall complete the transition by the end of the period of performance of the Task Order.
- (h) The transition-out plan shall address, at a minimum, the following areas:
 - Inventory and orderly transfer of all government furnished equipment/government furnished property (GFE/GFP);
 - Briefing on all in-progress and committed items; and
 - Any additional information required by other clauses contained in the contract.

No later than fifteen (15) days after notification of a transition-out date, the contractor shall provide a complete Incumbent Staff List to the COR and CO. This list must include names, position, start date (at the performance location), and contact information for each employee currently performing services under the incumbent order.

10. ATTACHMENTS

- A. Contract Discrepancy Report
- B. Medical Staffing Site Status Report Master Template
- C. Advanced Practice Practitioner (APP) Collaborative Practice and Prescriptive Authority Agreement or similar Component agreement

Performance Requirements Summary

FUNCTIONAL AREA	ACCEPTABLE QUALITY LEVEL AND PERFORMANCE STANDARD	WITHHOLDING/DEDUCTION CRITERIA
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	<i>Note: The referenced policies and procedures establish a minimum and maximum for the acceptable quality level and/or performance standard. These levels and/or frequency shall be further established at the order level based on order level requirements.</i> <i>Represented functional area acceptable quality levels below may be included in full or in part at the order level based on order level requirements.</i>	
Staffing	Adequate staffing is critical to ensuring continuity of care and meeting patient needs. Contractors must maintain at least 90% of available staffing levels, filling vacancies based upon Component requirements and processing standards, unless the COR authorizes otherwise. Contractor will provide a monthly staffing report using the Medical Staffing SSR Master Template to include incumbent positions, vacancies, number/type/percentage of State credentialed positions in travel status (e.g., local position or traveler positions with licensure portability for travel mobility), and availability to work (e.g., full-time/part-time, active/inactive, and clearances of delayed, full or limited Background Investigation) by performance location and positions.	A Contract Discrepancy Report that cites violations of cited standards or work statement sections that provide satisfactory staffing levels that permits the Contracting Officer to withhold or deduct up to 20% of a month invoice until the Contract Officer determines there is full compliance with the standard or section.
Retention	High staff retention promotes efficacy in the continuity of care, compliance to Government policies, and minimizes security risk. Contractor must maintain an annual retention rate of $\geq 85\%$ for full-time staff. Contractor must report quarterly retention metrics to the COR. The 85% threshold balances operational stability with industry averages.	A Contract Discrepancy Report that cites violations of standards or work statement sections that permits the Contract Officer to withhold or deduct up to 20% of a monthly invoice until the

		Contract Officer determines there is full compliance with the standard or section.
Turnover	Excessive turnover disrupts clinical operations, creates training burden, and undermines quality. Contractor's annual turnover rate may not exceed 15% of staff (excluding government-directed removals or approved reassignments). Limiting turnover to 15% annually (excluding government-initiated removals) supports the Government's goal for stable workforce operations.	A Contract Discrepancy Report that cites violations of standards or work statement sections that maintain an orderly environment with clear expectations of behavior and systems of accountability permits the Contract Officer to withhold or deduct up to 10% of a monthly invoice until the Contract Officer determines there is full compliance with the standard of section.
Backfill plan	Performance locations cannot tolerate prolonged staffing gaps. Contractors must implement and document contingency plans, providing qualified backfill for key positions identified by the Component within 5–10 business days of a vacancy or extended absence. Contingency staffing plans must be documented and provided to COR.	Deductions apply when coverage lapses exceed the timeframe without COR-approved justifica-

		tion. 10% deduction per incident where coverage exceeds 10 business days without approved justification.
Supplies/ Equipment/ Formulary	Uninterrupted access to necessary supplies, functional medical equipment, and pharmaceutical/medication for PIC and anyone requiring emergent medical care. Contractors are responsible for ensuring 100% availability of critical medical supplies and functioning equipment as defined in this document or resulting work statement and DHS and Component standards, with inventory reconciled monthly. Contractor maintains an effective and efficient inventory management strategy to ensure the 100% availability of formularies and supplies is readily available when needed and purchased at a reasonable price. Contractor enacts and administers proper and secure medical waste disposal strategy at performance locations, in compliance with all federal, state and local regulations, as applicable.	Deductions apply when shortages or equipment failures impede care delivery. 20% deduction per incident of unavailable/deficient supply and/or equipment directly impacting patient care or delaying services.
Quality in Scope of Care Services	Addresses service provider responsibility to provide for the basic needs and personal care of aliens within national and agency-accepted standards and using medical quality assurance methods to improve on patient safety and reduction in medical risks. Contractor shall ensure all aliens and persons in custody are provided quality medical care consistent with current nationally accepted medical standards at the federal, state, DHS and Component levels, as applicable, on medical protocols, directives, issuances, processes, and quality management practices as outlined in the contract PWS and applicable government furnished Medical Quality Management (MQM) plan and implementation information attachments. • Contractor shall implement a comprehensive MQM plan and oversight that adheres to nationally recognized industry standards. • Contractor shall proactively conduct medical quality checks, assess medical incident reviews, and track risk and quality management processes across all positions and sites to address medical quality, safety, and incident gaps and improvements needs in response actions. • Contractor shall prepare risk and activity and assurance report. • Contractor shall promote, share best practices, and conduct training to consistently carry out the activities indicated in the MQM and Quality Assurance/Patient Safety Risk Management Plan/Report	Failure to comply with requirements can result in significant risk to alien health, and liability, and therefore may warrants higher deduction penalties if unmet. A Contract Discrepancy Report that cites violations of standards or work statement sections that provide for the basic needs and personal care of persons in custody, permits

	and in alignment with the government medical quality guidance, protocols, and processes. • Contractor shall provide a medical quality surveillance report per Component requirements to the agency Medical Quality, Safety and/or Medical Program Manager, to include identification and action plans for medical incident and near miss medical risks and safety incidents to improve employee quality and scope of care practice. • Contractor shall utilize telehealth solutions, mobile diagnostics, and digital scheduling platforms where appropriate to increase access to clinical care and services, as well as enhance operational efficiency. A minimum 95% compliance rate on government COR audits and inspections and 100% in quarterly closure rates on findings and risks courses of action planned monthly, based on a monthly MQM and Quality Assurance/Patient Safety Risk Management plan/report to mitigate risks from audits, safety, and inspection findings.	the Contract Officer to withhold or deduct up to 25% of a monthly invoice until the Contract Officer determines there is full compliance with the standard or section.
Start date	Timely contract initiation ensures that care is not disrupted. Delays in staff reporting create immediate operational gaps that compromise access to medical services. All contractor staff must report for duty and be available to perform services on the contractually agreed start date unless a COR-approved delay is granted. 100% compliance is required.	10% deduction per staff member per day of delay beyond approved start date.

Orientation, Training, and Competency	Mandatory orientation, training, and annual assessments maintain staff readiness, reduce clinical errors, and uphold national, DHS, and Component standards for quality of care and patient safety. Training must cover the difference between medical assessments, medical encounters, and reasons for the difference. Contractor must ensure all staff complete initial and annual competency assessments consistent with national, DHS, and Component standards (e.g., Basic life support (BLS), Cardiopulmonary resuscitation (CPR), Health insurance portability and accountability act (HIPAA), Prison rape elimination act (PREA), suicide prevention, emergency response, infection control, high-risk individuals). Contractor shall perform a quarterly review of their employees to ensure compliance with work statement and professional standards with input from the government COR. Contractors maintain a professional and courteous bearing while performing their duties. Any substantiated and documented cases of unprofessional contractor behavior at any time is resolved by contract management a timely and professional manner. Contractor shall develop an orientation/training program for all staff. All new staff must complete all necessary orientation within 5 days after reporting to the performance location. If required, orientation can be completed at the performance location by their designated POC. Orientation and refresher shall provide information on the scope of practice, teaching manuals, any necessary medical and policy related documents, as well as information on component system, training, and reporting requirements. Contractor shall provide monthly reporting of training updates provided to employees to refresh or align updated training on any updated terms, policies, procedures, system, and guidance to be provided to the COR and Component Medical Quality and Program Manager. 100% compliance is required. Documentation is required.	Staff member not current on mandatory training will be prohibited from working.
Collaborative Practice Agreement (CPA)	Ensures legal compliance for advanced practice providers and safe clinical oversight, reducing liability and ensuring adherence to scope of practice laws. Advanced practice providers [i.e., nurse practitioners (NPs) and physician assistants (PAs)] and/or key component designated supervising positions must maintain current collaborative practice agreements (CPAs) or similar Component agreement with supervising physicians in accordance with state law and agency's standards, agreements, and requirements, as applicable.	Staff member will be prohibited from working.

	100% current CPA compliance is required. The CPA shall include a Quality Assurance/Patient Safety Risk Management Plan approach to describe how the contractor shall monitor, lower and manage patient safety risk, to include data/information on risk management policies that support and promote DHS and Component policies. The CPA shall also describe how the contractor implements the measures necessary to meet prevailing federal and state patient safety guidelines, as applicable, and proactively administer a risk and incident management program tailored to agency directives, processes and guidance and how it is applied consistently throughout all the performance locations. The provider's signed Component Medical Unit checklist shall accompany the signed CPA, where required by Component guidelines.	
Credentialing	Verification of credentials protects persons in custody from receiving care from unqualified healthcare providers, mitigates liability, and aligns with federal healthcare compliance requirements. All staff must be properly credentialed prior to providing care, with verification of education, training, licensure, DEA, and references per credentialing policy. Contractor shall provide Credentialing Letter from the Component Medical Quality & Risk Reduction program leads and provide monthly verification and validation of vetting status on any change and updates to the currency and active licensure/credentials compliance to federal, state and DHS licensure requirements, as applicable. Contractor shall monitor and report the status of credentials of all healthcare providers to the COR on a continuous basis (a minimum of monthly but to not exceed quarterly) to ensure all licensed contract staff remain in compliance with credentialing and licensure requirements.	CDR and reimbursement to the government on hours worked per staff member who provides services without valid credentials.
Privileging	Privileges confirm that providers are authorized and competent to deliver specific services, reducing risk of malpractice and ensuring quality standards. All licensed independent providers (LIPs) must be granted and maintain clinical privileges through the appropriate agency privileging process before initiating patient care.	CDR and reimbursement to the government on hours worked per staff member who provides services without approved privileges.

Documentation & Reporting	Accurate documentation ensures continuity of care, enables accountability, timely transfer of clinical information, and supports privacy and legal compliance with federal, DHS, and Component standards, Contractor shall use designated DHS Electronic Medical Record (EMR) systems. Contractor shall provide quarterly summaries of focused medical quality management chart reviews to the COR and Program Manager and/or designated medical lead. Reporting should include recommendations for process and quality improvement, where appropriate or identified by the contractor. Contractor staff must complete all required medical documentation, system or records entries, incident reports, mortality reviews) within DHS and Component timelines (24 hours for routine, immediate for critical incidents). Contractor shall document and report significant events related to the health of a person in custody while in custody at a performance location where medical contracted support is present. Contractor shall require a written sentinel event report and situation report (SITREP) be provided to the COR and Program Manager. The SITREP shall be provided within three (3) hours from the time the contractor learns of the event. • The sentinel event report, analyzing root causes, shall be provided within a 24-hour period and may be edited as more information becomes available. Immediately following notification of a sentinel event, the medical service provider shall provide telephonic and/or email notification of the event to the COR and Program Manager. • The contractor shall also provide all medical records related to the associated with sentinel event to the COR and Program Manager within three (3) hours of the time the contractor learns of the sentinel event.	5% deduction per occurrence of missing, delayed, or incomplete documentation.
Compliance & Audits	Contractor shall conduct quarterly compliance reviews and audits. Contractor shall be accountable for performance, reduce risk of systemic failures, and align with DHS and Component oversight expectations. Contractor shall achieve $\geq 95\%$ compliance during COR/DHS/Component audits, inspections, assessments, and compliance reviews. Contractor shall provide a quarterly risk management report to the COR identifying risk factors and addressing all compliance, audits, and assessment incidents organized by performance locations and positions	10% deduction if compliance falls below threshold in quarterly audit.

	impacted in the risk assessment and mitigation, to include risks to persons in custody, performance location operations, and agency operations.	
Emergency Response	Participation in drills ensures staff readiness for medical emergencies, disasters, and crises, protecting life and minimizing risk. Contractor must ensure staff participation in medical emergency response and drills per performance location, DHS, and Component COOP plan. 100% staff participation is required.	10% deduction per occurrence of non-response or failure to participate in required drills.
Medical Liability Insurance Coverage	Contractor shall provide a certificate of medical liability malpractice insurance. Minimum coverage shall be \$1 million per occurrence and \$2 million per aggregate. 100% compliance is required.	Staff member will be prohibited from working.